

The Over-the-Counter Body

Edition 2 in the Philips series. Previous Decision Intelligence report: 'Hospital Goes Home, Home Goes Medical' (June 2026). Research question: what medical devices do people buy and use at home to track, treat or prevent health issues?

Cycle: 3 August 2026 · Audience: Executive Committee, Personal Health and Sleep & Respiratory Care leadership · Horizon: to 2030

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● risk ● opportunity ● mixed/neutral impact × likelihood: — high — med — low



What you'll learn

1. Why the medical device is becoming a retail purchase: over-the-counter glucose monitors, a watch that flags blood pressure and sleep apnea, and earbuds that work as hearing aids, after the FDA cut red tape in January 2026.
2. Why home devices are climbing from tracking to treating and preventing, with at-home diagnostics, longevity testing, home therapy and remote monitoring now reimbursed.
3. Why sleep is the beachhead category, and why clinical validation and trust, sharpened by Philips' own Respironics recall, decide who wins it.
4. Why the home is turning AI-native and ambient, and why the real contest to 2030 is Big Tech platforms against clinical incumbents for the home-health layer.

Key takeaways

1. In January 2026 the FDA said single-output decision-support software needs no approval and physiological-sensing wearables can sell as wellness tools without clearance, opening the gates to a wearable medical-device market put at 68 billion dollars in 2026 and 330 billion by 2033.
2. Devices are moving up the clinical value chain: the self-test market reaches about 22 billion dollars by 2033, Function Health raised 450 million dollars for at-home blood biomarkers, and CMS eased remote-monitoring billing from 1 January 2026.
3. Sleep is the highest-volume home-medical category, a 34.6 billion dollar sleep-tech market in 2026, where consumer watches and rings now flag apnea as triage, and where clinical validation and Philips' 1.1 billion dollar Respiroics settlement make trust the differentiator.
4. The home is turning AI-native: Google launched a 9.99-dollar-a-month AI health coach, Big Tech contests the consumer-health layer, and 71 percent of consumers find AI personalisation intrusive, so validated, trusted devices become the moat.

Executive Synthesis

Why this matters now

The medical device is leaving the clinic. In the first half of 2026 a person could buy a continuous glucose monitor off a shelf, be told by a watch that they may have high blood pressure or sleep apnea, and use their earbuds as a hearing aid, all without a prescription. In January 2026 the US FDA cut red tape on consumer wearables and decision-support software, and the consumer flood accelerated. For Philips, whose portfolio spans consumer Personal Health and clinical Sleep and Respiratory Care, the question this cycle is not whether the home becomes a point of care, the evidence settles that, but where in that migration Philips competes, and whether trust, the very thing its Respiroics recall put in question, becomes the moat or the exposure.

Four shifts this cycle records, and the pattern that names them:

- **Monitoring goes over the counter.** Glucose, blood pressure, heart rhythm, apnea and hearing move from prescription to retail, and the device you buy now makes a clinical claim.
- **Devices climb from tracking to treating and preventing.** At-home diagnostics, longevity testing, home therapy and reimbursed remote monitoring pull diagnosis and care into the home.
- **Sleep is the beachhead.** The highest-volume home-medical category is where consumer and clinical devices collide, and where trust decides the winner.
- **The home turns AI-native and contested.** AI coaching and ambient sensing make monitoring passive, and consumer-platform giants contest the layer with medtech.

This cycle names the variable that will decide Philips' position in 2030: not whether care migrates home, but who owns the home-health layer, the consumer-platform giants or the clinical incumbents, and whether validated trust commands a premium or is commoditised away. Four developments carry the pattern.

First, monitoring has gone over the counter. On 6 January 2026 the FDA said clinical-decision-support software offering a single recommendation can reach market without approval, and physiological-sensing wearables can sell as wellness tools without clearance, a move a leading law firm read as [widening the general-wellness carve-out](#) and reversing a 2025 warning to WHOOP over blood pressure ([STAT News](#)). Around it, over-the-counter

glucose sensors scaled: Dexcom redesigned its [Stelo over-the-counter platform](#) and posted [1.31 billion dollars in Q2 2026](#), while Apple's hypertension and sleep-apnea notifications and AirPods hearing aid reached [over 160 countries](#). A study found self-fitted [AirPods Pro matched fitted hearing aids](#). The wearable medical-device market is put at [68 billion dollars in 2026](#), rising to [330 billion by 2033](#).

Second, devices are climbing from tracking to treating and preventing. The self-test market is forecast to reach [about 22 billion dollars by 2033](#), blood-based cancer screening is [scaling through national lab networks](#), and the American Cancer Society now [recommends at-home stool and blood tests](#). Deloitte finds [76 percent of providers expect patient-initiated testing to reshape diagnostics](#). Consumer longevity testing has gone mainstream, with Function Health raising [450 million dollars](#), and home therapy is proliferating, from [red-light therapy](#) to [cuffless blood-pressure systems](#). Critically, CMS [eased remote-monitoring billing from 1 January 2026](#) and Congress [extended hospital-at-home through 2030](#), so reimbursement now follows care into the home.

Third, sleep is the beachhead. It is the highest-volume home-medical category, a [34.6 billion dollar sleep-tech market in 2026](#), where consumer watches and rings now flag apnea. Apple's algorithm reaches [66 percent sensitivity and 98.5 percent specificity](#), positioning wearables as triage, not diagnosis, and Samsung is extending apnea detection to its [Galaxy Watch and the first over-the-counter smart ring](#). This is Philips' home ground, and its most exposed: the Respironics recall is [99 percent remediated](#) by mid-2026, but the [roughly 1.1 billion dollar personal-injury settlement](#) still awaits approval and US sales of new sleep devices remain gated by a [consent decree](#). GLP-1 drugs, feared as a CPAP threat, in fact [lift PAP initiation](#).

Fourth, the home is turning AI-native and contested. Google launched a [9.99-dollar-a-month AI health coach](#), Apple [pared back its own](#), and Oura and [Samsung push predictive rings](#). Contactless [radar sensing](#) and the [smart-ring market, to 7.8 billion dollars by 2035](#) make monitoring passive. But the layer is contested: a peer-reviewed review counts [five Big Tech consumer health-AI assistants](#), and analysts argue [Big Tech now owns the consumer-health layer](#). The AI-in-healthcare market reaches [744 billion dollars by 2035](#). Yet [71 percent of consumers find AI personalisation intrusive](#), and [only 34 percent say their doctor uses their wearable data](#).

The device now makes the claim the clinic used to make, and the only defensible difference left is whether anyone can prove it works.

The historical rhyme worth holding is the shift from the film camera to the phone: the incumbents owned the optics and the chemistry, but the platform that owned the everyday device owned the category. In consumer health the everyday device is the phone, the watch and the ring, and the platforms that own them now own continuous health signal. Philips owns the clinical grade and the regulatory scar tissue. The prudent posture is to treat over-the-counter monitoring, the move up the value chain, the sleep franchise and the AI-native home as one connected strategy question, not four product decisions.

Where this analysis could be wrong

The most consequential assumption is that the FDA's January 2026 deregulation holds and consumer devices keep winning clinical ground. If a high-profile failure, a wearable that misses a serious event, or a data breach triggers a regulatory snap-back, the advantage swings hard to validated, cleared, clinical-grade devices, which is Philips' strength. The falsifiable tests: whether the FDA reverses or narrows the wellness carve-out before 2028; whether a consumer-device accuracy scandal reaches Congress; and whether payers begin refusing consumer-generated data. Signals to watch: an FDA enforcement action against a cleared-adjacent wearable, a payer coverage policy on consumer data, and the first class action over a missed diagnosis by a wellness wearable.

What this cycle establishes

1. **Over the counter.** Monitoring is now a retail purchase and the FDA has stepped back. The open variable is whether the carve-out holds or snaps back after an incident.
2. **Up the value chain.** Diagnostics, therapy and prevention are moving home with reimbursement behind them. The open variable is how fast payers and clinicians accept home-generated data.
3. **Sleep is the beachhead.** The category is contested and trust-sensitive, and Philips is both incumbent and scarred. The open variable is when Philips can re-enter US sleep at scale.
4. **AI-native and contested.** The home is turning passive and Big Tech owns the everyday device. The open variable is whether Philips competes, partners or supplies the clinical layer beneath the platforms.
5. **Name the pattern: the clinic came to the counter.** The device makes the claim, so the strategy question shifts from what to build to what to prove, and to who owns the layer.

Each is developed below, with a decision posture, in the four Strategic Implications.

Continuity ledger: how the June cycle's six forces have moved

The June edition, "Hospital Goes Home, Home Goes Medical", named six forces behind the hospital-to-home convergence. Two have hardened, two have escalated, one is recast into this cycle's sleep-trust theme, and one carries forward. This cycle sharpens the read from provider-side hospital-at-home to the consumer-purchased device that now makes the clinical claim.

June force	Status	2030 read
Hospital-at-Home Crosses the Reimbursement Threshold	ESCALATED	Congress extended the hospital-at-home waiver five years to 2030 , moving this from threshold to durable policy runway.
Reimbursement Categories Are Rewiring What Gets Built	CONFIRMED	CMS eased remote-monitoring billing from January 2026 ; reimbursement now follows care into the home, carried into this cycle's value-chain theme.
Aging Demographics Make Domestic Health a Mass Market	CONFIRMED	Ambient and aging-in-place monitoring are spreading, with contactless sensing evidence maturing ; domestic health is now a mass consumer market, not a niche.
AI Reshapes the Device Form Factor	ESCALATED	The home is turning AI-native, from Google's 9.99-dollar AI health coach to predictive rings; this cycle isolates it as the platform contest.
CMF and Materials Under Regulatory Pressure	RECAST	The materials-and-quality question is now the trust the sleep category must rebuild: Respironics is 99 percent remediated with a settlement pending, recast into this cycle's sleep theme.
Patient Experience as Clinical Outcome	CARRIED	Validation and trust now differentiate; the National Sleep Foundation cautions consumer tech supports better sleep only with limits , carrying the experience-as-outcome read forward.

Audience Snapshots

Four functional lenses on the same intelligence base. Each card names the shift and the single question this cycle puts to that part of Philips.

PERSONAL HEALTH (CONSUMER PRODUCT LEADERSHIP)

Are you competing on the device, or on the claim the device can prove?

The shift: monitoring is now a retail purchase, the FDA has [stepped back](#), and the wearable medical-device market is put at [68 billion dollars in 2026](#), with Big Tech owning the everyday device.

The question to brief: should Philips compete head-on in over-the-counter monitoring, or concentrate on validated, clinical-grade devices and services where proof, not price, wins?

SLEEP & RESPIRATORY CARE

Is the sleep franchise re-anchored on trust, or still defined by the recall?

The shift: sleep is the beachhead category, a [34.6 billion dollar market](#), consumer watches now [triage apnea](#), the recall is [99 percent remediated](#), and GLP-1 drugs [lift PAP starts](#).

The question to brief: when and how does Philips re-enter US sleep at scale, and can clinical validation turn the Respironics scar into a trust advantage?

REGULATORY, QUALITY & CLINICAL AFFAIRS

Is the January 2026 deregulation a threat to plan around, or a moat to build?

The shift: the FDA [un-gated decision-support software and wellness wearables](#) in January 2026, while the [National Sleep Foundation warns consumer devices cannot substitute for clinical evaluation](#).

The question to brief: should Philips make FDA clearance and clinical validation an explicit market signal, and prepare for the snap-back scenario an incident could trigger?

DIGITAL, DATA & AI

Are you the platform, or the pipe beneath someone else's platform?

The shift: the home is turning AI-native, Google launched a [9.99-dollar AI coach](#), [Big Tech owns the consumer-health layer](#), and [71 percent of consumers find AI personalisation intrusive](#).

The question to brief: should Philips build a consumer health-data and AI layer, partner into the platforms, or supply the clinical-grade layer beneath them?

Themes

The cycle's signals are organised into four themes, ranked by impact on Philips' near-term decisions.

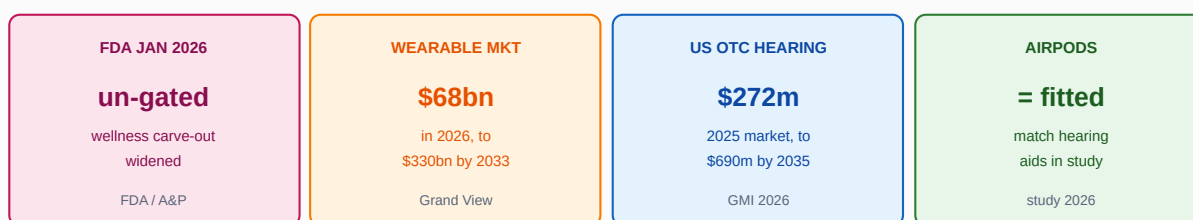
Immediate: shapes portfolio and positioning decisions now. **Near-Term**: shapes competitive position over the next year or two. **Longer-Range**: a multi-year shift to track each cycle.

1. The Over-the-Counter Clinic: consumer devices win clinical claims as the FDA cuts red tape

IMMEDIATE

Clinical monitoring has become a retail purchase. On 6 January 2026 the FDA said single-output decision-support software needs no approval and physiological-sensing wearables can sell as wellness tools without clearance, and the consumer flood accelerated: over-the-counter glucose sensors, a watch that flags blood pressure and sleep apnea, and earbuds cleared as hearing aids. The device now makes the claim the clinic used to make, and it is sold without a prescription. For Philips the gate is open to everyone at once.

Monitoring goes over the counter, in four numbers



The over-the-counter shift in four numbers. Source: FDA via STAT and Arnold & Porter, Grand View Research, Global Market Insights, peer-reviewed study via Yahoo Health.

- On 6 January 2026 the FDA said single-recommendation decision-support software can reach market without approval and physiological-sensing wearables can sell as wellness tools without clearance. [STAT News](#) (6 January 2026).
- A law-firm analysis reads the move as widening the general-wellness carve-out to non-invasive sensing wearables and reversing a 2025 warning to WHOOP over blood-pressure monitoring. [Arnold & Porter](#) (21 January 2026).
- Dexcom posted 1.31 billion dollars in Q2 2026, up 13 percent, scaling its prescription and over-the-counter Stelo glucose lines. [MassDevice](#) (30 July 2026); the earlier redesign of the [Stelo platform](#) added AI insights (Drug Delivery Business, 30 April 2026).
- Apple's hypertension notifications, sleep-apnea notifications and AirPods Pro hearing aid reached over 160 countries by May 2026. [9to5Mac](#) (21 May 2026); the FDA cleared the [hypertension feature](#) to flag over a million undiagnosed people (MobiHealthNews).
- A 118-person study found self-fitted 249-dollar AirPods Pro matched professionally fitted hearing aids with no adverse events, in a market where about 75 percent of adults with hearing loss go untreated. [study via Yahoo Health](#) (5 July 2026).
- The US over-the-counter hearing-aids market was 272.1 million dollars in 2025 and is forecast to reach 690.8 million by 2035, a 9.3 percent CAGR. [Global Market Insights](#) (February 2026).
- British Columbia became the first Canadian province to allow over-the-counter hearing aids in 2026, with AirPods Pro 3 at 329 dollars against prescription devices at 1,000 to 4,000. [MobileSyrup](#)

(3 July 2026).

- The global wearable medical-devices market is estimated at 68.1 billion dollars in 2026 and projected to reach 330.5 billion by 2033, a 29.5 percent CAGR. [Grand View Research](#) (June 2026).

Counter-argument

The carve-out could snap back before it commoditises the category. The FDA move rests on enforcement discretion, not statute, and a single high-profile failure or a change of administration could narrow the general-wellness exemption as fast as it widened, swinging the advantage to cleared, validated devices. The falsifiable test is whether the FDA reverses or narrows the carve-out before 2028. Actors to watch: the FDA, Congress, and the first litigant over a missed diagnosis.

Weak signals to watch

- **WEAK SIGNAL** Retail and pharmacy chains could become the primary channel for clinical monitoring. Would gain weight if a major retailer launches a private-label OTC monitoring line.
- **WEAK SIGNAL** The 25 percent revenue-share and licensing template the FDA is exploring for some devices could reshape margins. Would gain weight if a second consumer-health category adopts a fee-for-claim model.

Decision link: Strategic Implications [1](#) and [3](#).

2. From Tracking to Treating and Preventing: home devices climb the clinical value chain

NEAR-TERM

Home devices are climbing from tracking to treating and preventing. What began as step-counting now spans at-home diagnostics, blood-based screening, consumer longevity panels, home therapy and remote monitoring, and reimbursement has followed: CMS eased remote-monitoring billing from January 2026 and Congress extended hospital-at-home to 2030. The value is migrating from the wrist to the whole care pathway, and the home is becoming a site of diagnosis and treatment, not just measurement.

Up the value chain, in four numbers



The value-chain climb in four numbers. Source: Grand View Research, MobiHealthNews, CMS via McDonald Hopkins, Deloitte via HIT Consultant.

- The global self-testing market was 11.8 billion dollars in 2025 and is projected to reach 21.9 billion by 2033, an 8.5 percent CAGR. [Grand View Research](#) (June 2026).
- Guardant's FDA-approved Shield blood test for colorectal cancer became orderable across roughly 2,000 Quest patient service centres, citing 93 percent screening adherence and Medicare coverage. [Guardant via Business Wire](#) (10 March 2026).
- The American Cancer Society updated its colorectal guidelines to include at-home stool and blood tests, stressing that the most effective test is the one the patient completes. [American Cancer Society](#) (27 May 2026).
- Deloitte found 76 percent of providers expect patient-initiated testing to fundamentally alter diagnostics within three years, and 92 percent already offer or are exploring it. [Deloitte via HIT Consultant](#) (29 May 2026).
- Function Health raised 450 million dollars to expand its at-home blood-biomarker platform, after a 298 million dollar round at a 2.5 billion dollar valuation. [MobiHealthNews](#) (31 July 2026); the [longevity-diagnostics market](#) reaches 4.54 billion by 2033 (Grand View Research).
- At-home therapy is proliferating, from a [red-light-therapy market to 1.13 billion dollars by 2033](#) to Hilo's [cuffless blood-pressure system at 89.99 dollars](#) capturing 30 to 35 readings a day (PR Newswire, 21 July 2026).
- Effective 1 January 2026 CMS added remote-monitoring codes billable for just 2 to 15 days of data and 10 minutes of management, down from 16 days and 20 minutes. [McDonald Hopkins](#) (5

November 2025).

- Congress extended the CMS Acute Hospital Care at Home waiver five years through 2030, after it had been set to expire in January 2026. [American Medical Association](#) (2 March 2026).

Counter-argument

The value-chain climb could stall at the clinician's desk. Home-generated data still meets scepticism: only about a third of patients say their doctor uses their wearable data, and reimbursement, while easing, remains fragmented across categories. If clinicians and payers do not accept home diagnostics and therapy at scale, the market stays consumer-paid and shallow rather than clinical and deep. The falsifiable test is whether payer coverage of consumer-generated data broadens before 2028. Actors to watch: CMS, commercial payers, and specialty societies.

Weak signals to watch

- **WEAK SIGNAL** At-home blood biomarker panels could become an annual consumer ritual like a dental check. Would gain weight if a large employer adds home biomarker testing to benefits.
- **WEAK SIGNAL** Digital therapeutics could bundle with home-monitoring hardware into a subscription. Would gain weight if a device maker launches a reimbursed hardware-plus-therapy subscription.

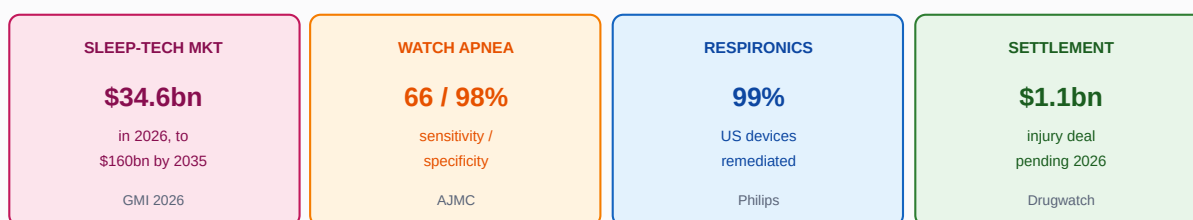
Decision link: Strategic Implications [2](#) and [1](#).

3. Sleep as the Beachhead: apnea detection, CPAP and the trust the category must rebuild

NEAR-TERM

Sleep is the beachhead. It is the highest-volume home-medical category and the place where consumer and clinical devices collide first: a watch or a ring now flags apnea to hundreds of millions, while CPAP and diagnostic sleep testing remain clinical and reimbursed. It is also Philips' home ground and its deepest scar. The Respironics recall is all but remediated, but the settlement is unresolved and US re-entry is gated, so in the one category Philips knows best, trust is both the battleground and the liability.

Sleep as the beachhead, in four numbers



The sleep beachhead in four numbers. Source: Global Market Insights, American Journal of Managed Care, Philips Respironics, Drugwatch.

- The sleep-tech devices market is estimated at 34.6 billion dollars in 2026, up from 29.3 billion in 2025, and forecast to reach 160.6 billion by 2035, an 18.6 percent CAGR. [Global Market Insights](#) (July 2026).
- The FDA-cleared Apple Watch sleep-apnea algorithm shows 66.3 percent sensitivity but a deliberately high 98.5 percent specificity, positioning wearables as triage rather than a replacement for polysomnography. [American Journal of Managed Care](#) (22 June 2026).
- Samsung's Galaxy Watch carries FDA-cleared sleep-apnea detection reaching general availability on 7 August 2026, and the Galaxy Ring will become the first over-the-counter FDA-cleared smart ring with apnea risk detection. [HME News](#) (23 July 2026); Apple's feature is live in [over 150 countries](#) (Apple).
- As of July 2026 Philips reports 2.686 million US sleep-therapy devices remediated, about 99 percent of actionable global registrations, and 97,000 of 111,000 US ventilators remediated. [Philips Respironics](#) (15 July 2026).
- As of February 2026 Philips maintains its recalled devices are not expected to cause appreciable harm and may only resume US sleep sales once 2024 consent-decree requirements are met. [Koninklijke Philips N.V.](#) (10 February 2026).
- The roughly 1.1 billion dollar Philips personal-injury settlement still awaits final court approval, atop a 479 million dollar economic-loss settlement and a roughly 400 million dollar consent decree. [Drugwatch](#) (1 July 2026).

- ResMed found from a 2.1-million-patient analysis that sleep-apnea patients on GLP-1 drugs were 1,060 basis points more likely to start PAP therapy, so the drugs expand rather than erode CPAP demand. [ResMed via Investing.com](#) (30 April 2026).
- The global sleep-apnea devices market grows from 8.37 billion dollars in 2025 to 9.05 billion in 2026, reaching 12.54 billion by 2030. [The Business Research Company](#) (July 2026).

Counter-argument

Consumer apnea flags could enlarge the clinical funnel rather than cannibalise it. A watch that triages apnea sends more people, not fewer, toward diagnosis and CPAP, and GLP-1 drugs are already lifting PAP initiation. If the consumer wave grows the diagnosed population, Philips' clinical franchise benefits from the very devices that look like competitors. The falsifiable test is whether diagnosed-OSA and PAP-initiation rates rise alongside consumer apnea features through 2027. Actors to watch: ResMed, sleep-medicine societies, and Philips' own re-entry timeline.

Weak signals to watch

- **WEAK SIGNAL** A consumer smart ring could win first-line apnea screening before Philips re-enters US sleep. Would gain weight if an over-the-counter ring secures a broad apnea-screening indication.
- **WEAK SIGNAL** The Respireonics settlement's final terms could reset Philips' cost of re-entry. Would gain weight if the court approves or rejects the personal-injury settlement in 2026.

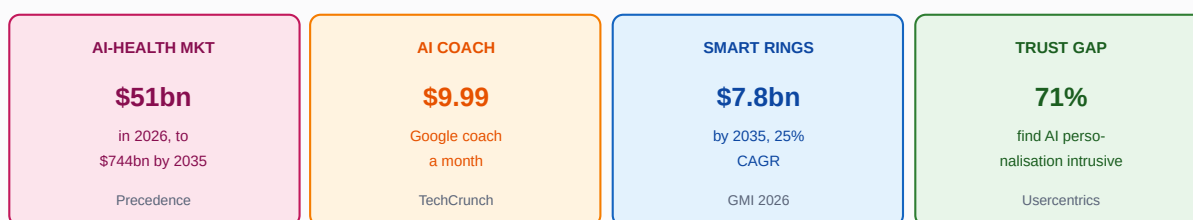
Decision link: Strategic Implications [3](#) and [4](#).

4. The Ambient, AI-native Home: passive sensing, AI coaching and the platform contest

LONGER-RANGE

The home is turning AI-native and ambient, and the layer is contested. AI coaching turns raw data into guidance, contactless radar and rings make monitoring passive, and the everyday device, the phone, the watch and the ring, is owned by Apple, Samsung and Google, not by medtech. The strategic contest to 2030 is not about a better sensor but about who owns the home-health layer and the continuous signal it generates, and whether a scarred, validated incumbent can be the trusted clinical grade beneath, or beside, the platforms.

The AI-native home, in four numbers



The AI-native home in four numbers. Source: Precedence Research, TechCrunch, Global Market Insights, Usercentrics.

- Google's Gemini-powered AI health coach launched on 19 May 2026 at 9.99 dollars a month inside Google Health Premium, initially for Fitbit and Pixel Watch users. [TechCrunch](#) (7 May 2026).
- Apple scaled back Project Mulberry, its AI health coach and redesigned Health app, after leadership judged it did not beat rivals such as Oura and Whoop. [9to5Mac](#) (5 February 2026).
- Samsung confirmed a Galaxy Ring 2 and a Vitals baseline feature, with an AI health-coaching system targeted for 2027 that its digital-health head says could predict behaviour with 70 to 80 percent accuracy. [Forbes](#) (25 June 2026).
- A peer-reviewed review catalogues seven unobtrusive home-sensing modalities, including radar for contactless vital-sign extraction, and flags accuracy, privacy and acceptance as the barriers. [Biosensors, MDPI](#) (29 April 2026).
- The smart-ring market was about 1.01 billion dollars in 2026 and is forecast to reach 7.8 billion by 2035, a 25.4 percent CAGR. [Global Market Insights](#) (April 2026).
- A JMIR viewpoint profiles five newly consumer-facing Big Tech health-AI assistants launched 2025 to early 2026, noting Amazon's 200-plus clinics and a Microsoft diagnostic model at 80 percent accuracy against 20 percent for physician panels. [Journal of Medical Internet Research](#) (30 April 2026).
- Analysts argue Big Tech now owns healthcare's infrastructure, AI and consumer layers, with wearables creating continuous longitudinal signal that displaces episodic clinical data. [Galen](#)

Growth (June 2026); the [AI-in-healthcare market reaches 744 billion dollars by 2035](#) (Precedence Research).

- Consumer trust is the brake: 71 percent find AI personalisation intrusive and 52 percent would pay about 7 percent more for transparency, while [only 34 percent say their doctor uses their wearable data](#). [Usercentrics](#) (29 June 2026).

Counter-argument

The platform contest could resolve into partnership, not conquest. Apple pared back its own AI coach, the platforms lack clinical-grade validation and regulatory standing, and consumers distrust AI with their health data. If Big Tech needs a trusted clinical layer it cannot build cheaply, the incumbent that owns validation and regulatory credibility becomes the partner of choice rather than the casualty. The falsifiable test is whether a major platform signs a clinical-grade medtech partnership before 2028. Actors to watch: Apple, Samsung, Google, and the medtech incumbents.

Weak signals to watch

- **WEAK SIGNAL** A trust backlash could hand validated incumbents a marketing edge overnight. Would gain weight if a consumer-health data breach reaches mainstream headlines.
- **WEAK SIGNAL** Ambient radar sensing could leapfrog wearables for older adults who will not wear a device. Would gain weight if a contactless home monitor secures reimbursement for aging-in-place.

Decision link: Strategic Implication [4.](#)

Strategic Implications

Four decisions this cycle puts to Philips, each with a posture, an owner and the themes it draws on.

SI 1: Decide where Philips competes in the over-the-counter monitoring wave

Monitoring is now a retail purchase, the FDA has [stepped back](#), and the everyday device belongs to Big Tech, with the wearable market put at [68 billion dollars in 2026](#). Philips cannot out-scale Apple or Samsung on consumer volume, but it can own the tier where a claim must be proven. Decide explicitly: compete in commodity monitoring, or concentrate on validated, clinical-grade devices, services and the trust position, and treat the consumer wave as a funnel into that tier rather than a market to win outright.

Action: this cycle, set a board-level position on which consumer-monitoring categories Philips competes in directly versus cedes, and make clinical validation the explicit differentiator.

Who gains: the incumbents that pick a defensible tier early, and the clinical and regulatory teams whose validation becomes a market signal rather than a cost.

DECIDE

Draws on Themes [1](#) and [3](#). Owner: Executive Committee / Personal Health leadership.

SI 2: Prepare the move up the value chain from tracking to home diagnosis and therapy

Devices are climbing from measurement to [diagnostics](#), [biomarkers](#) and [therapy](#), and reimbursement now follows with [eased remote-monitoring billing](#) and [hospital-at-home to 2030](#). Philips' clinical credibility is an asset precisely where consumer brands are weakest: proven diagnosis and treatment at home. Prepare a home diagnostics-and-therapy path, organically or by partnership, that pairs Philips' clinical grade with the reimbursement now moving into the home. If clinician acceptance lags, the capability is still a hedge against a consumer-only market.

Action: within two cycles, define a home diagnosis-and-therapy roadmap and a reimbursement-tracking capability tied to the 2026 CMS and hospital-at-home changes.

Who gains: the operators that pair clinical validation with home reimbursement early, and the care-pathway partners that win the home referral.

PREPARE

Draws on Theme 2. Owner: Connected Care / Personal Health with Strategy.

SI 3: Prepare to re-anchor the sleep franchise on trust, and time US re-entry

Sleep is the beachhead, the recall is [99 percent remediated](#), the [1.1 billion dollar settlement](#) is unresolved, and US sleep sales remain [gated by a consent decree](#), even as consumer devices [triage apnea](#) and GLP-1 drugs [lift PAP starts](#). Prepare to turn the Respiroics scar into a trust position: lead re-entry with clinical validation and quality transparency, and time it to the settlement and consent-decree milestones. If a consumer ring wins first-line screening before re-entry, Philips still owns the diagnosed, treated tier the triage feeds.

Action: this cycle, set a trust-led US sleep re-entry plan with milestones tied to the consent decree and settlement, and a clinical-validation message for the category.

Who gains: the franchise that reframes remediation as proof of quality, and the clinical channel that captures the enlarged, triaged funnel.

PREPARE

Draws on Theme 3. Owner: Sleep & Respiratory Care with Regulatory & Quality.

SI 4: Monitor the platform contest, and decide compete, partner or supply

The home is turning AI-native, [Big Tech owns the consumer-health layer](#), Google runs a [9.99-dollar AI coach](#), yet [71 percent distrust AI personalisation](#) and the platforms lack clinical-grade validation. Monitor whether the contest resolves into conquest or partnership, and pre-position for the likeliest outcome: supplying the trusted clinical layer beneath, or beside, the platforms. Do not commit capital to a consumer AI-coaching platform of Philips' own until the partner-versus-compete signal resolves.

Action: monitor for a major platform-plus-medtech clinical partnership and a consumer-health trust event, and hold an option to supply the clinical layer rather than build a rival platform.

Who gains: the incumbent that keeps its validation and data credibility as leverage, and avoids a losing platform war it does not need to fight.

MONITOR

Draws on Theme 4. Owner: Digital, Data & AI with Corporate Strategy.

Scenario Matrix

Four operating environments for Philips to 2030, generated by crossing two axes the evidence does not yet decide: who owns the home-health layer, clinical incumbents or Big Tech platforms, and whether the regulatory environment stays deregulated or scrutiny returns after an incident. The matrix is a planning tool, not a forecast; the value sits in the indicators that would tip Philips from one cell to another.

HOME-HEALTH LAYER (CLINICAL INCUMBENTS TO BIG TECH PLATFORMS)

The Trust Premium

The best pairing for Philips: clinical incumbents hold the home-health layer and regulatory scrutiny returns, so validated, cleared devices command a premium. The Respironics scar, converted into demonstrable quality, becomes an advantage, and consumer triage feeds Philips' diagnosed, treated tier. Proof, not price, wins.

Indicators: the FDA narrows the wellness carve-out after an incident; payers require validated data; Philips completes a trust-led US sleep re-entry.

Platforms Under the Microscope

Big Tech owns the everyday device but scrutiny returns, so the platforms hold reach while validation and regulatory standing become the scarce input. Philips' opening is to supply the trusted clinical layer beneath the platforms, the partner of choice for claims the platforms cannot themselves prove.

Indicators: a platform faces an FDA action or data-breach headline; a major platform signs a clinical-grade medtech partnership; consumer trust tightens.

The Open Field

Clinical incumbents hold the layer and deregulation persists, so Philips can move fast, but so can every medtech rival and every well-funded entrant. Speed, distribution and reimbursement capture decide share, and the trust advantage is real but not yet priced by a cautious regulator.

Indicators: the carve-out holds; home diagnostics and therapy scale on reimbursement; competition is medtech-versus-medtech rather than platform-led.

SCRUTINY

Commoditised and Sidelined

The worst pairing: Big Tech owns the layer and deregulation persists, so monitoring commoditises, the platforms own the continuous signal, and clinical validation earns no premium in a market that no longer asks for proof. Philips is squeezed to a shrinking prescription tier while the value accrues to whoever owns the device and the data.

Indicators: the carve-out widens further; platforms bundle monitoring free; payers accept consumer data without validation; Philips' consumer share erodes.

DEREGULATED

What We Are Not Planning For

Four scenarios held out of the plan because the evidence does not yet justify resourcing against them. Each carries the reinstatement trigger that would change that judgement.

Exclusion 1: A wholesale FDA re-regulation reversing the January 2026 deregulation

The [January 2026 carve-out](#) rests on enforcement discretion and could narrow, but a full statutory reversal within the horizon is not the base case. We are not planning for a wholesale snap-back, while treating a targeted narrowing after an incident as a live scenario (see the Trust Premium cell).

Reinstatement trigger: *an FDA enforcement action against a cleared-adjacent wellness wearable, or Congressional legislation restoring pre-2026 oversight.*

Exclusion 2: Consumer devices fully replacing clinical diagnosis

Wearable apnea and hypertension features are [triage tools](#), high in specificity but modest in sensitivity, and the [National Sleep Foundation warns they cannot substitute for clinical evaluation](#). We are not planning for consumer devices to replace clinical diagnosis to 2030; they enlarge the funnel into it.

Reinstatement trigger: *an over-the-counter device securing a broad diagnostic, not screening, indication that payers accept in place of clinical testing.*

Exclusion 3: GLP-1 drugs collapsing the CPAP and sleep-apnea device market

The evidence points the other way: GLP-1 patients are [more likely to start PAP therapy](#), and the [sleep-apnea device market keeps growing](#). We are not planning for a GLP-1-driven collapse of the PAP market within the horizon.

Reinstatement trigger: *multi-year evidence that GLP-1 uptake reduces diagnosed-OSA or PAP-initiation rates at population scale.*

Exclusion 4: Philips exiting consumer and personal health entirely

The question this cycle is where Philips competes, not whether it participates; the consumer wave is a funnel into Philips' clinical tier, not only a threat. We are not planning for a full exit from consumer and personal health, while treating selective retreat from commodity monitoring as a legitimate option under SI 1.

***Reinstatement trigger:** a board decision to divest personal health, or sustained evidence that no defensible consumer tier remains.*

Discussion Points

1. If monitoring is now a retail purchase and the FDA has stepped back, do we compete in over-the-counter devices, or concentrate on the validated, clinical-grade tier where proof wins?
2. **For Sleep & Respiratory Care:** can we turn the Resironics scar into a trust advantage, and when do we re-enter US sleep at scale against consumer devices already triaging apnea?
3. If devices are climbing from tracking to treating and preventing with reimbursement behind them, do we build a home diagnosis-and-therapy path now, or wait for clinician acceptance to firm up?
4. **For Digital, Data & AI:** are we the platform, the partner into the platforms, or the trusted clinical layer beneath them, and what would tip that choice?
5. If validated trust is Philips' distinctive asset, do we make FDA clearance and clinical proof an explicit market signal now, before a snap-back or a trust event forces the issue?

Source Confidence Register

40 verified sources across four themes. Tier 1 (governments, regulators, multilateral bodies): 0. Tier 2 (institutional research, market-research firms, peer-reviewed journals, professional societies, law-firm and consultancy analyses): 21. Tier 3 (specialist trade and quality press): 17. Tier 4 (company and vendor primary, used for named launches only): 2. This is a consumer-trends cycle, so the evidence base leans on institutional research (Tier 2) and specialist trade press (Tier 3) rather than government primary; 21 of 40 sources (52%) sit at Tier 1 or 2. Recency window is 180 days from the 3 August 2026 cycle; 4 sources are structural anchors just outside it (the January 2026 FDA action, the CMS 2026 rule and the Apple hypertension clearance) that define the cycle, used as context. The source set was frozen at 2026-08-03. Detect → Assess → Decide → Act.

Theme 1: The Over-the-Counter Clinic

Source	Tier	Date	Key claim
Arnold & Porter	T2	21 Jan 2026	The FDA introduced enforcement discretion for single-output decision-support software and widened the general-wellness carve-out to non-invasive physiological-sensing wearables, reversing its July 2025 warning to WHOOP over blood-pressure monitoring. (<i>structural anchor</i>)
Global Market Insights	T2	15 Feb 2026	The US over-the-counter hearing-aids market was valued at 272.1 million dollars in 2025 and is forecast to reach 690.8 million dollars by 2035, a 9.3 percent CAGR.
Grand View Research	T2	1 Jun 2026	The global wearable medical devices market is estimated at 68.1 billion dollars in 2026 and projected to reach 330.5 billion dollars by 2033, a 29.5 percent CAGR.
MobiHealthNews	T3	15 Sep 2025	Apple received FDA clearance for an Apple Watch hypertension-notification feature expected to flag over one million people with undiagnosed hypertension in its first year across 150-plus countries. (<i>structural anchor</i>)
STAT News	T3	6 Jan 2026	On 6 January 2026 the FDA said clinical-decision-support software giving a single recommendation can reach market without approval, and wearables tracking blood pressure, glucose or heart rate can be sold as wellness tools without clearance. (<i>structural anchor</i>)

Source	Tier	Date	Key claim
Drug Delivery Business News	T3	30 Apr 2026	Dexcom reported Q1 2026 sales of 1.19 billion dollars, up 15 percent, and announced a complete redesign of its Stelo over-the-counter glucose platform with AI-driven personalised insights.
9to5Mac	T3	21 May 2026	By May 2026 Apple's hypertension notifications, sleep-apnea notifications and AirPods Pro hearing-aid feature were live in over 160 countries, with fresh rollouts in Taiwan and India.
MobileSyrup	T3	3 Jul 2026	British Columbia became the first Canadian province to allow over-the-counter hearing aids in 2026, with AirPods Pro 3 at 329 Canadian dollars against prescription devices at 1,000 to 4,000 dollars.
Yahoo Health	T3	5 Jul 2026	A 118-person study found self-fitted 249-dollar AirPods Pro matched professionally fitted hearing aids with no adverse events, in a market where about 75 percent of adults with age-related hearing loss remain untreated.
MassDevice	T3	30 Jul 2026	Dexcom posted Q2 2026 revenue of 1.31 billion dollars, up 13.1 percent, lifting full-year guidance as its prescription and over-the-counter Stelo glucose-sensor lines scale.

Theme 2: From Tracking to Treating and Preventing

Source	Tier	Date	Key claim
American Medical Association	T2	2 Mar 2026	The Consolidated Appropriations Act, 2026 extended the CMS Acute Hospital Care at Home waiver by five years through 2030, after it had been set to expire at the end of January 2026.
Grand View Research	T2	1 May 2026	The global longevity-diagnostics market was 2.80 billion dollars in 2025 and is projected to reach 4.54 billion dollars by 2033 at a 6.38 percent CAGR.

Source	Tier	Date	Key claim
American Cancer Society	T2	27 May 2026	Updated American Cancer Society colorectal guidelines now include at-home stool tests and blood tests, stressing that the most effective screening test is the one the patient completes.
Grand View Research	T2	1 Jun 2026	The global self-testing market was 11.8 billion dollars in 2025 and is projected to reach 21.9 billion dollars by 2033 at an 8.5 percent CAGR.
Grand View Research	T2	1 Jun 2026	The red-light-therapy market was 533.8 million dollars in 2025 and is forecast to reach 1.13 billion dollars by 2033 at a 9.8 percent CAGR, as at-home devices proliferate.
McDonald Hopkins	T3	5 Nov 2025	Effective 1 January 2026 CMS added remote-monitoring codes billable for just 2 to 15 days of data and 10 minutes of management, down from the prior 16-day and 20-minute minimums. <i>(structural anchor)</i>
HIT Consultant (Deloitte)	T3	29 May 2026	Deloitte found 76 percent of provider respondents say consumer- and patient-initiated testing will fundamentally alter diagnostics within three years, and 92 percent already offer or are exploring patient-initiated testing.
MobiHealthNews	T3	31 Jul 2026	Function Health raised 450 million dollars to expand its at-home blood-biomarker platform, following a 298 million dollar Series B at a 2.5 billion dollar valuation.
Guardant Health (Business Wire)	T4	10 Mar 2026	Guardant's FDA-approved Shield blood test for colorectal cancer became orderable across Quest's roughly 2,000 patient service centres, citing a 93 percent screening-adherence rate and Medicare coverage.
Hilo (PR Newswire)	T4	21 Jul 2026	Hilo launched a cuffless wearable plus FDA-cleared connected-cuff blood-pressure system at 89.99 dollars hardware and 149.99 dollars a year, capturing 30 to 35 readings a day including overnight data.

Theme 3: Sleep as the Beachhead

Source	Tier	Date	Key claim
Koninklijke Philips N.V.	T2	10 Feb 2026	As of 10 February 2026 Philips maintains its recalled CPAP and BiPAP devices are not expected to result in appreciable harm, and may only resume US sales of new sleep devices once the 2024 consent-decree requirements are met.
National Sleep Foundation (Univ. of Miami)	T2	25 Mar 2026	The 2026 National Sleep Foundation position statement concludes consumer sleep technology can support sleep habits but cannot substitute for clinical evaluation and warns of orthosomnia, sleep anxiety driven by device data.
American Journal of Managed Care	T2	22 Jun 2026	The FDA-cleared Apple Watch sleep-apnea algorithm shows 66.3 percent sensitivity but a deliberately high 98.5 percent specificity, positioning wearables as triage tools rather than replacements for polysomnography.
The Business Research Company	T2	1 Jul 2026	The global sleep-apnea devices market is projected to grow from 8.37 billion dollars in 2025 to 9.05 billion dollars in 2026, reaching 12.54 billion dollars by 2030 at an 8 percent CAGR.
Global Market Insights	T2	1 Jul 2026	The sleep-tech devices market is estimated at 34.6 billion dollars in 2026, up from 29.3 billion in 2025, and forecast to reach 160.6 billion dollars by 2035 at an 18.6 percent CAGR.
Philips Respironics	T2	15 Jul 2026	As of July 2026 Philips reports 2.686 million US sleep-therapy devices remediated, about 99 percent of actionable global registrations, and 97,000 of 111,000 US ventilators remediated.
Investing.com (ResMed)	T3	30 Apr 2026	ResMed posted Q3 FY26 revenue of 1.43 billion dollars and found from a 2.1-million-patient analysis that sleep-apnea patients on GLP-1 drugs were 1,060 basis points more likely to start PAP therapy, suggesting the drugs expand rather than erode CPAP demand.

Source	Tier	Date	Key claim
Apple Newsroom	T3	21 May 2026	Apple Watch sleep-apnea notifications are now available in more than 150 countries using an accelerometer-based algorithm that analyses breathing disturbances every 30 days.
Drugwatch	T3	1 Jul 2026	As of July 2026 the roughly 1.1 billion dollar Philips personal-injury settlement still awaits final court approval, atop a separate 479 million dollar economic-loss class settlement and a roughly 400 million dollar FDA consent decree.
HME News	T3	23 Jul 2026	Samsung's Galaxy Watch carries FDA-cleared sleep-apnea detection reaching general availability on 7 August 2026, and the Galaxy Ring will become the first over-the-counter FDA-cleared smart ring with sleep-apnea risk detection later in 2026.

Theme 4: The Ambient, AI-native Home

Source	Tier	Date	Key claim
Global Market Insights	T2	1 Apr 2026	The smart-ring market was 697.6 million dollars in 2025, about 1.01 billion in 2026, and is forecast to reach 7.8 billion dollars by 2035 at a 25.4 percent CAGR.
Biosensors (MDPI)	T2	29 Apr 2026	A peer-reviewed review catalogues seven unobtrusive home-sensing modalities, including radar for contactless vital-sign extraction, and flags accuracy, privacy and user acceptance as the barriers to ambient home monitoring.
Journal of Medical Internet Research	T2	30 Apr 2026	A JMIR viewpoint profiles five newly consumer-facing Big Tech health-AI assistants launched 2025 to early 2026, noting Amazon's 200-plus clinic footprint and a Microsoft diagnostic model at 80 percent accuracy against 20 percent for physician panels.
Precedence Research	T2	25 May 2026	The global AI-in-healthcare market was 36.96 billion dollars in 2025, about 51.20 billion in 2026,

Source	Tier	Date	Key claim
			and is projected to reach 744.34 billion dollars by 2035 at a 35.02 percent CAGR.
Galen Growth	T2	15 Jun 2026	Galen Growth argues Big Tech now owns healthcare's infrastructure, AI and consumer layers, with Apple's wearables creating continuous longitudinal health signals that displace episodic clinical data.
Usercentrics	T2	29 Jun 2026	Usercentrics' 2026 survey of 11,000 consumers found 71 percent find AI-driven personalisation intrusive and 47 percent took a revenue-impacting action over data concerns, while 52 percent would pay about 7 percent more for AI transparency.
EY	T2	21 Jul 2026	EY's 2026 US survey found 55 percent would request tests or treatment based on AI-tool information, 76 percent see AI or search as reliable health sources, but only 34 percent say their doctor uses their wearable data.
9to5Mac	T3	5 Feb 2026	Apple scaled back Project Mulberry, its AI health coach and redesigned Health app, after leadership judged it did not beat rivals such as Oura and Whoop, slipping features toward iOS 27.
TechCrunch	T3	7 May 2026	Google's Gemini-powered AI health coach launched on 19 May 2026 at 9.99 dollars a month inside Google Health Premium, initially for Fitbit and Pixel Watch users.
Forbes	T3	25 Jun 2026	Samsung confirmed a Galaxy Ring 2 and a Vitals baseline feature, with an AI health-coaching system targeted for 2027 that its digital-health head says could predict behaviour with 70 to 80 percent accuracy within two to three years.

Claim-Fidelity Appendix: Analyst inferences and editorial framing

This briefing carries analyst-generated interpretations that go beyond what any single source asserts. They are named here so a reader can trace the confidence line and disagree productively.

Analytical inferences carried by the body prose

- "The clinic came to the counter" is an analyst frame linking four independently sourced threads, the FDA deregulation, over-the-counter monitoring, the value-chain climb and the AI-native home, into one pattern; no single source asserts the composite.
- Casting trust and clinical validation as Philips' defensible moat is an analyst judgement built from the FDA carve-out, the National Sleep Foundation caution and the consumer-distrust data; it is a strategic reading, not a claim any source makes about Philips specifically.
- Reading the platform contest as likely to resolve into partnership rather than conquest is an interpretation of Apple's scaled-back coach, the platforms' missing clinical validation and consumer distrust; the opposite outcome is disclosed in the Commoditised and Sidelined scenario.
- The film-camera-to-phone analogy is an interpretive overlay chosen to discipline the platform-ownership thesis, not a claim the sources make.
- Treating consumer apnea features as enlarging rather than cannibalising Philips' clinical funnel is an analyst inference from the triage framing and the GLP-1 PAP-initiation data; it is contested in the same theme's counter-argument.

Editorial framing

The title "The Over-the-Counter Body" is an editorial compression: it names the through-line that a person's vital signs, from glucose to blood pressure to apnea to hearing, are now measurable through devices bought without a prescription. As the first edition in the Philips series, this report sets a baseline rather than grading a prior cycle, so it carries no continuity ledger. Figures drawn from market-research firms are directional forecasts, cited to a single provider each and flagged as such; where a market number is load-bearing it is corroborated by a named launch or a regulatory fact rather than resting on the forecast alone.

The Futures Wheel and the in-body figure SVGs are analyst constructions built from the sourced claims; they carry no separate source-register entry, so a claim or quantification flag on those elements is expected and disclosed here rather than being a provenance failure.